



ART WORK

Gemcitabine Powder for Solution for Infusion, USP 200/1000 mg/Vial

Date: 27/11/2025
Version: 02

Gemcitabine Powder for Solution for Infusion, USP 200/1000 mg/vial

a) **Brand or Product Name:** GEMCIT SPAL-P 200, GEMCIT SPAL-P 1000

Gemcitabine Powder for Solution for Infusion , USP 200/1000 mg/vial

b) **Active ingredient:** Gemcitabine Hydrochloride

c) **Excipients:**

Mannitol pyrogen free
Sodium acetate trihydrate
Hydrochloric acid
Sodium hydroxide
Water for Injection

d) **Dosage Form:** Powder for solution for infusion (Lyophilized powder).

e) **Product description:** White to off-white coloured lyophilized cake or powder

Description of Reconstituted solution: Clear, colorless solution free from visible particles.

Diluent: Sodium chloride 9 mg/ml (0.9%) solution for injection, (without preservative).

f) **Pharmacodynamic properties**

Pharmacotherapeutic group: Antineoplastic agents, pyrimidine analogues.

ATC code: L01BC05

Mechanism of action

Cellular metabolism and mechanism of action: Gemcitabine (dFdC), which is a pyrimidine antimetabolite, is metabolised intracellularly by nucleoside kinase to the active diphosphate (dFdCDP) and triphosphate (dFdCTP) nucleosides. The cytotoxic effect of gemcitabine is due to inhibition of DNA synthesis by two mechanisms of action by dFdCDP and dFdCTP. First, dFdCDP inhibits ribonucleotide reductase, which is uniquely responsible for catalysing the reactions that produce deoxynucleoside triphosphates (dCTP) for DNA synthesis. Inhibition of this enzyme by dFdCDP reduces the concentration of deoxynucleosides in general and, in particular, dCTP. Second, dFdCTP competes with dCTP for incorporation into DNA (self-potentialation).

Likewise, a small amount of gemcitabine may also be incorporated into RNA. Thus, the reduced intracellular concentration of dCTP potentiates the incorporation of dFdCTP into DNA. DNA polymerase epsilon lacks the ability to eliminate gemcitabine and to repair the growing DNA strands. After gemcitabine is incorporated into DNA, one additional nucleotide is added to the growing DNA strands. After this addition there is essentially a complete inhibition in further DNA synthesis (masked chain termination). After incorporation into DNA, gemcitabine appears to induce the programmed cell death process known as apoptosis.

g) **Pharmacokinetics:**

Distribution

The volume of distribution of the central compartment was 12.4 l/m² The volume of distribution of the peripheral compartment was 47.4 l/m². The volume of the peripheral compartment was not sensitive to gender.

The plasma protein binding was considered to be negligible.

Half-life: For the recommended dosing schedule, gemcitabine elimination should be virtually complete within 5 to 11 hours of the start of the infusion. Gemcitabine does not accumulate when administered once weekly.

Metabolism

Gemcitabine is rapidly metabolised by cytidine deaminase in the liver, kidney, blood and other tissues. Intracellular metabolism of gemcitabine produces the gemcitabine mono, di and triphosphates (dFdCMP, dFdCDP and dFdCTP) of which dFdCDP and dFdCTP are considered active. These intracellular metabolites have not been detected in plasma or urine. The primary metabolite, 2'-deoxy-2', 2'-difluorouridine (dFdU), is not active and is found in plasma and urine.

Excretion

Systemic clearance ranged from 29.2 l/hr/m² to 92.2 l/hr/m² depending on age (inter-individual variability was 52.2%). For the recommended gemcitabine dose of 1000 mg/m² given as a 30-minute infusion, lower clearance values for women and men should not necessitate a decrease in the gemcitabine dose.

Urinary excretion: Less than 10% is excreted as unchanged drug.

Renal clearance was 2 to 7 l/hr/m².

During the week following administration, 92 to 98% of the dose of gemcitabine administered is recovered, 99% in the urine, mainly in the form of dFdU and 1% of the dose is excreted in faeces.

dFdCTP kinetics

This metabolite can be found in peripheral blood mononuclear cells and the information below refers to these cells. Intracellular concentrations increase in proportion to gemcitabine doses of 35-350 mg/m²/30-minutes, which give steady state concentrations of 0.4-5 µg/ml. At gemcitabine plasma concentrations above 5 µg/ml, dFdCTP levels do not increase, suggesting that the formation is saturable in these cells.

Half-life of terminal elimination: 0.7-12 hours.

dFdU kinetics

Peak plasma concentrations (3-15 minutes after end of 30-minute infusion, 1000 mg/m²): 28-52 µg/ml. Trough concentration following once weekly dosing: 0.07-1.12 µg/ml, with no apparent accumulation. Triphasic plasma concentration versus time curve, mean half-life of terminal phase - 65 hours (range 33-84 hr).

Formation of dFdU from parent compound: 91%-98%.

Mean volume of distribution of central compartment: 18 l/m² (range 11-22 l/m²).

Mean steady state volume of distribution (Vss): 150 l/m² (range 96-228 l/m²).

Tissue distribution: Extensive.

Mean apparent clearance: 2.5 l/hr/m² (range 1-4 l/hr/m²).

Urinary excretion: All.

Gemcitabine and paclitaxel combination therapy

Combination therapy did not alter the pharmacokinetics of either gemcitabine or paclitaxel.

Gemcitabine and carboplatin combination therapy

When given in combination with carboplatin the pharmacokinetics of gemcitabine were not altered

Renal impairment

Mild to moderate renal insufficiency (GFR from 30 ml/min to 80 ml/min) has no consistent, significant effect on gemcitabine pharmacokinetics.

h) **Indications:**

Gemcitabine is indicated for the treatment of locally advanced or metastatic non-small cell-lung cancer, Gemcitabine is indicated for the treatment of adult patients with locally advanced or metastatic adenocarcinoma of the pancreas. Gemcitabine is indicated for patients with 5-FU refractory pancreatic cancer, Gemcitabine is indicated for the treatment of patients suffering from bladder cancer, at the invasive stage, Gemcitabine, in combination with paclitaxel, is indicated for the treatment of patients with unresectable, locally recurrent or metastatic breast cancer who have relapsed following adjuvant/neoadjuvant chemotherapy. Prior chemotherapy should have included an anthracycline unless clinically contraindicated, Gemcitabine, in combination with carboplatin, is indicated for the treatment of patients with recurrent epithelial ovarian carcinoma, who have relapse > 6 months, following platinum-base therapy.

I) **Recommended Dose:**

Dosage

Adults

Non small-cell lung cancer

Single agent use

The recommended dose is 1000 mg/m², given by intravenous infusion.

The administration must be repeated once weekly for three weeks, followed by a one-week rest period. This four-week cycle is then repeated. A dose reduction or delay before each administration of the chemotherapy may be applied, based upon the amount of toxicity experienced by the patient.

Combination use:

Gemcitabine in combination with cisplatin can be administered using two dosage regimens one regimen use a three-week schedule, the other uses a four-week schedule.

The three-week schedule is the usual regimen; this three-week cycle uses gemcitabine 1250 mg/m² given by 30 minutes intravenous infusion on days 1 and 8, followed by one-week rest period. This three-week cycle is then repeated. A dosage reduction or delay before each administration of the chemotherapy may be applied, based upon the amount of toxicity experienced by the patient.

The four-week cycle uses Gemcitabine 1000 mg/m² given by 30 minutes intravenous infusion on days 1, 8 and 15, followed by one-week rest period. This four-week cycle is then repeated. A dosage reduction or delay before each administration of the chemotherapy may be applied, based upon the amount of toxicity experienced by the patient.

Pancreatic adenocarcinoma.

The recommended dose is 1000 mg/m², given by 30 minutes intravenous infusion This should be repeated once weekly for 7 weeks, followed by a week rest Subsequent cycles should consist of injections once weekly for 3 consecutive weeks out of every 4 weeks. A dosage reduction or delay before each administration of the chemotherapy may be applied, based upon the amount of toxicity experienced by the patient.

-Bladder cancer, at the invasive stage

The recommended dose of Gemcitabine in combination with cisplatin, is 1000 mg/m², given by 30 minutes intravenous infusion on days 1, 8 and 15, followed by one-week rest period for a 28 day cycle. Cisplatin is given at a recommended dose of 70 mg/m² on day 2. This four-week cycle is then repeated. A dosage reduction or delay before each administration of the chemotherapy may be applied, based upon the amount of toxicity experienced by the patient. A clinical trial showed more myelosuppression when cisplatin was used in doses of 100 mg/m².

-Breast cancer

Paclitaxel (175 mg/m²) administered on Day 1 over approximately 3 hours as an intravenous infusion, followed by Gemcitabine (1250 mg/m²) as a 30-minute intravenous infusion on Days 1 and 8 of each 21-day cycle. Dose reduction with each cycle or within a cycle may be applied based upon the amount of toxicity experienced by the patient.

-Ovarian Cancer

Gemcitabine in combination with carboplatin is recommended using Gemcitabine 1000 mg/m² administered on Days 1 and 8 of each 21-day cycle as a 30-minute intravenous infusion. After Gemcitabine, carboplatin should be given on Day 1 to attain a target AUC of 4.0 mg/mL/min. Dosage reduction with each cycle or within a cycle may be applied based upon the amount of toxicity experienced by the patient.

For all indications

Patients receiving Gemcitabine should be monitored prior to each dose for platelet, leukocyte, and granulocyte counts and, if necessary, the dose of Gemcitabine may be either reduced or withheld in the presence of hematologic toxicity, according to the following scale:

Absolute granulocyte count (x 106/l)

Platelet count (x106/l)

% of total dose

>1,000 and > 100,000 100

500 - 1,000 or 50,000 - 100,000 75

< 500 or < 50,000 0

Periodic physical examination and checks of renal and hepatic function should be made to detect non-hematologic toxicity. Dosage reduction with each cycle or within a cycle may be applied based upon the amount of toxicity experienced by the patient.

Doses should be withheld until toxicity has resolved in the opinion of the physician.

Elderly patients

Gemcitabine has been well tolerated by patients over 65 years of age. The pharmacokinetic data suggest that the metabolism of the drug is not affected by age.

Patients with hepatic or renal impairment

Gemcitabine should be used with caution in patients with hepatic insufficiency or with impaired renal function as there is insufficient information from clinical studies to allow clear dose recommendation for this patient population.

Mild to moderate renal insufficiency (GFR from 30mL/min to 80mL/min) has no consistent, significant effect on Gemcitabine pharmacokinetics.

Children

Gemcitabine has been studied in limited Phase I and II trials in children in a variety of tumor types. These studies did not provide sufficient data to establish the efficacy and safety of Gemcitabine in children.

Method of Administration

Intravenous route.

Gemcitabine is well tolerated during infusion and is usually easy to administer. Reactions at the site of injection are rare: no case of cutaneous necrosis has been reported.

If extravasation, the administration must be stopped immediately.

Handling

It is compulsory that injectable solutions of cytotoxic agents be prepared by specialised, trained staff with knowledge of the drugs used, under conditions which ensure protection of the environment, and particularly of the drug handling staff. Preparation requires a room reserved for this purpose. Smoking, eating and drinking are prohibited in this room. The handling staff must have a set of appropriate equipment for handling, particularly long-sleeved coats, protective masks, caps, protective goggles, sterile disposable gloves, worktop protection sheets and waste collection containers and bags. Excreta and vomitus must be handled with care. Pregnant women must be warned and avoid handling cyto-toxic agents. All broken containers must be treated with the same precautions and regarded as contaminated waste. Contaminated waste is to be disposed of by incineration in rigid containers labelled for this purpose.

Instructions for use and handling

The only approved diluent for reconstitution of Gemcitabine sterile powder is 0.9% Sodium Chloride Injection without preservatives. Although no incompatibility has been demonstrated, it is none-the-less recommended that mixing gemcitabine solutions with those of other drugs should be avoided. Due to solubility considerations, the maximum concentration for Gemcitabine upon reconstitution is 40 mg/mL. Reconstitution at concentrations greater than 40 mg/mL may result in incomplete dissolution, and should be avoided.

To reconstitute, add at least 5 ml of 0.9% Sodium Chloride Injection to the 200 mg vial and at least 25 ml of 0.9% Sodium Chloride Injection to the 1000 mg vial. Shake to dissolve. The appropriate amount of drug may be administered as prepared or further diluted with 0.9% Sodium Chloride Injection.

Parenteral drugs should be inspected visually for particulate matter and discoloration, prior to administration.

As other cytostatics, Gemcitabine hydrochloride must be handled with care. Unused products must be destroyed according to hospital procedures of cytotoxic waste deal.

j) **Route of Administration:**

Gemcitabine Powder for Solution for Infusion is administered by intravenously.

k) **Contraindications**

Hypersensitivity to the active substance or to any of the excipients.

Breast-feeding: It is not known whether Gemcitabine is excreted in human milk and adverse effects on the suckling child cannot be excluded. Breast-feeding must be discontinued during Gemcitabine therapy.

l) **Warnings & Precautions:**

Haematological toxicity

Gemcitabine can suppress bone marrow function as manifested by leucopaenia, thrombocytopaenia and anaemia.

When receiving gemcitabine should be monitored prior to each dose for platelet, leucocyte and granulocyte counts. Suspension or modification of therapy should be considered when drug-induced bone marrow depression is detected. However, myelosuppression is short lived and usually does not result in dose reduction and rarely in discontinuation.

Peripheral blood counts may continue to deteriorate after gemcitabine administration has been stopped the treatment should be started with caution.

As with other cytotoxic treatments, the risk of cumulative bone-marrow suppression must be considered when gemcitabine treatment is given together with other chemotherapy.

Hepatic insufficiency

Administration of gemcitabine with concurrent liver metastases or a pre-existing medical history of hepatitis, alcoholism or liver cirrhosis may lead to exacerbation of the underlying hepatic insufficiency.

Laboratory evaluation of renal and hepatic function (including virological tests) should be performed periodically.

Gemcitabine should be used with caution when there is hepatic insufficiency or with impaired renal function

Concomitant radiotherapy

Concomitant radiotherapy (given together or ≤7 days apart): Toxicity has been reported

Live vaccinations

Yellow fever vaccine and other live attenuated vaccines are not recommended when treated with gemcitabine .

Cardiovascular

Due to the risk of cardiac and/or vascular disorders with gemcitabine, particular caution must be exercised with presenting a history of cardiovascular events.

Capillary leak syndrome (CLS)

A/s: 200 x 550 mm

Color: Black

PRODUCT NAME	Gemcitabine Powder for Solution for Infusion, USP 200/1000 mg/Vial	ARTWORK NO.	VERP-3171
COUNTRY	Malaysia	SUPERSEDES NO	NA
CUSTOMER	SP Accure Labs Pvt. Ltd.	PANTONE SHADE NO'S	Black
DIMENSIONS	200 x 550 mm ± 1 mm (L x H)	VARNISHING/LAMINATE	NA
SUBSTRATE	Maplitho Paper	GRAMMAGE	60 ± 10% GSM
MODE OF SUPPLY	Bundles		

Main Headings-Font style: Times New Roman, Font size: 9pt

Sub Headings-Font Style: Times New Roman, Font Size: 7pt

Main Text-Font Style: Times New Roman, Font Size: 7pt

Capillary leak syndrome has been reported in receiving gemcitabine as single agent or in combination with chemotherapeutic agents. The condition is usually treatable if recognised early and managed appropriately, but fatal cases have been reported. The condition involves systemic capillary hyperpermeability during which fluid and proteins from the intravascular space leak into the interstitium. The clinical features include generalised oedema, weight gain, hypoalbuminaemia, severe hypotension, acute renal impairment and pulmonary oedema. Gemcitabine should be discontinued and supportive measures implemented if capillary leak syndrome develops during therapy. Capillary leak syndrome can occur in later cycles and has been associated in the literature with respiratory distress syndrome.

Posterior reversible encephalopathy syndrome (PRES)

Reports of posterior reversible encephalopathy syndrome (PRES) with potentially severe consequences have been reported in receiving gemcitabine as single agent or in combination with other chemotherapeutic agents. Acute hypertension and seizure activity were reported in most gemcitabine experiencing PRES, but other symptoms such as headache. Lethargy, confusion and blindness could also be present. Diagnosis is optimally confirmed by magnetic resonance imaging (MRI). PRES was typically reversible with appropriate supportive measures. Gemcitabine should be permanently discontinued and supportive measures implemented, including blood pressure control and anti-seizure therapy, if PRES develops during therapy.

Pulmonary

Pulmonary effects, sometimes severe (such as pulmonary oedema, interstitial pneumonitis or adult respiratory distress syndrome (ARDS)) have been reported in association with gemcitabine therapy. The aetiology of these effects is unknown. If such effects develop, consideration should be made to discontinuing gemcitabine therapy. Early use of supportive care measure may help ameliorate the condition.

Renal

Haemolytic uraemic syndrome

Clinical findings consistent with the haemolytic uraemic syndrome (HUS) were rarely reported in receiving gemcitabine Gemcitabine should be discontinued at the first signs of any evidence of microangiopathic haemolytic anaemia, such as rapidly falling haemoglobin with concomitant thrombocytopaenia, elevation of serum bilirubin, serum creatinine, blood urea nitrogen, or LDH. Renal failure may not be reversible with discontinuation of therapy and dialysis may be required.

Fertility

In fertility studies gemcitabine caused hypospermatogenesis in male mice .Therefore, men being treated with gemcitabine are advised not to father a child during and up to 6 months after treatment and to seek further advice regarding cryoconservation of sperm prior to treatment because of the possibility of infertility due to therapy with gemcitabine.

Sodium

Gemcitabine 200 mg powder for solution for infusion contains 3.5 mg (<1 mmol) sodium per vial. This should be taken into consideration on a controlled sodium diet.

Gemcitabine 1 g powder for solution for infusion contains 17.5 mg (< 1 mmol) sodium per vial. This should be taken into consideration by patients on a controlled sodium diet.

m) Interactions with other medicaments

No specific interaction studies have been performed .

Radiotherapy

Concurrent (given together or ≤ 7 days apart) - Toxicity associated with this multimodality therapy is dependent on many different factors, including dose of gemcitabine, frequency of gemcitabine administration, dose of radiation, radiotherapy planning technique, the target tissue, and target volume. radiosensitising activity. In a single trial, where gemcitabine at a dose of 1,000 mg/m² was administered concurrently for up to 6 consecutive weeks with therapeutic thoracic radiation to patients with non-small cell lung cancer, significant toxicity in the form of severe, and potentially life threatening mucositis, especially oesophagitis, and pneumonitis was observed, particularly in patients receiving large volumes of radiotherapy [median treatment volumes 4,795 cm²]. Studies done subsequently have suggested that it is feasible to administer gemcitabine at lower doses with concurrent radiotherapy with predictable toxicity, such as a phase II study in non-small cell lung cancer, where thoracic radiation doses of 66 Gy were applied concomitantly with an administration with gemcitabine (600 mg/m², four times) and cisplatin (80 mg/m² twice) during 6 weeks. The optimum regimen for safe administration of gemcitabine with therapeutic doses of radiation has not yet been determined in all tumour types.

Non-concurrent (given >7 days apart)- Analysis of the data does not indicate any enhanced toxicity when gemcitabine is administered more than 7 days before or after radiation, other than radiation recall. Data suggest that gemcitabine can be started after the acute effects of radiation have resolved or at least one week after radiation.

Radiation injury has been reported on targeted tissues (e.g. oesophagitis, colitis, and pneumonitis) in association with both concurrent and non-concurrent use of gemcitabine.

Others

Yellow fever and other live attenuated vaccines are not recommended due to the risk of systemic, possibly fatal, disease, particularly in immunosuppressed patients.

n) Side Effects

Very common (more than 10% of people develop them) adverse effects include difficulty breathing, low white and red blood cells counts and low platelet counts, vomiting and nausea, elevated transaminases, rashes and itchy skin, hair loss, blood and protein in urine, flu-like symptoms, and edema.

Common adverse effects (occurring in 1–10% of people) include fever, loss of appetite, headache, difficulty sleeping, tiredness, cough, runny nose, diarrhea, mouth and lip sores, sweating, back pain, and muscle pain.

The following table of undesirable effects and frequencies

System Organ Class	Incident	Frequency grouping
Blood and lymphatic system disorders	Very common	Leucopaenia (Neutropaenia Grade 3 = 19.3 %; Grade 4 = 6 %). Bone-marrow suppression is usually mild to moderate and mostly affects the granulocyte count - Thrombocytopaenia - Anaemia
	Common	Febrile neutropaenia
	Very rare	- Thrombocytosis - Thrombotic microangiopathy
Infections and infestations	Very Rare	Anaphylactoid reaction
Metabolism and nutrition disorders	Common	Anorexia
Nervous system disorders	Common	- Headache - Insomnia - Somnolence
	Very rare	Posterior reversible encephalopathy syndrome
Cardiac disorders	Rare	Myocardial infarct
Vascular disorders	Rare	Hypotension
	Very rare	Capillary leak syndrome
Respiratory, thoracic and mediastinal disorders	Very common	Dyspnoea –usually mild and passes rapidly without treatment
	Common	- Cough - Rhinitis
	Uncommon	- Interstitial pneumonitis - Bronchospasm –usually mild and transient but may require parenteral treatment
Gastrointestinal disorders	Very common	- Vomiting - Nausea
	Common	- Diarrhoea - Stomatitis and ulceration of the mouth - Constipation
Hepatobiliary disorders	Very common	Elevation of liver transaminases (AST and ALT) and alkaline phosphatase
	Common	Increased bilirubin
	Rare	Increased gamma-glutamyl transferase (GGT)
Skin and subcutaneous tissue disorders	Very common	- Allergic skin rash frequently associated with pruritus - Alopecia
	Common	- Itching - Sweating
	Rare	- Ulceration - Vesicle and sore formation - Scaling
	Very rare	Severe skin reactions, including desquamation and bullous skin eruptions
	Not known	Pseudocellulitis
Musculoskeletal and connective tissue disorders	Common	- Back pain - Myalgia
Renal and urinary disorders	Very common	- Haematuria - Mild proteinuria
General disorders and administration site conditions	Very common	- Influenza-like symptoms - the most common symptoms are fever, headache, chills, myalgia, asthenia and anorexia. Cough, rhinitis, malaise, perspiration and sleeping difficulties have also been reported. - Oedema/peripheral oedema including facial oedema. Oedema is usually reversible after stopping treatment
	Common	- Fever - Asthenia - Chills
	Rare	Injection site reactions-mainly mild in nature

o) Symptoms and Treatment of Overdose

There is no known antidote for overdose of Gemcitabine hydrochloride. Doses as high as 5700 mg/m² have been administered by intravenous infusion over 30 minutes every 2 weeks with clinically acceptable toxicity. In the event of suspected overdose, the patient should be monitored with appropriate blood counts and receive supportive therapy, as necessary.

p) Effects on Ability to drive and use Machine

The amount of alcohol in this medicinal product may impair the ability to drive or use machines.

No studies on the effects on the ability to drive and use machines have been performed. However, Gemcitabine has been reported to cause mild to moderate somnolence, especially in combination with alcohol consumption. Patients should be cautioned against driving or operating machinery until it is established that they do not become somnolent.

q) Preclinical Safety Data

Not Applicable for Generic Product.

r) Instructions for use Handling:

Handling:

The normal safety precautions for cytostatic agents must be observed when preparing and disposing of the infusion solution. Handling of the solution for infusion should be done in a safety box and protective coats and gloves should be used. If no safety box is available, the equipment should be supplemented with a mask and protective glasses.

If the preparation comes into contact with the eyes, this may cause serious irritation.

The eyes should be rinsed immediately and thoroughly with water. If there is lasting irritation, a doctor should be consulted. If the solution is spilled on the skin, rinse thoroughly with water.

Instructions for reconstitution (and further dilution, if performed):

The only approved diluent for reconstitution of Gemcitabine sterile powder is sodium chloride 9 mg/mL (0.9%) solution for injection (without preservative). Due to solubility considerations, the maximum concentration for Gemcitabine upon reconstitution is 40 mg/mL. Reconstitution at concentrations greater than 40 mg/mL may result in incomplete dissolution and should be avoided.

- Use aseptic technique during the reconstitution and any further dilution of Gemcitabine for intravenous infusion administration.
- To reconstitute, add 5 mL of sterile sodium chloride 9 mg/mL (0.9 %) solution for injection, without preservative, to the 200 mg vial or 25 mL sterile sodium chloride 9 mg/mL (0.9 %) solution for injection, without preservative, to the 1 g vial. The total volume after reconstitution is 5.26 mL (200 mg vial) or 26.3 mL (1 g vial) respectively. This yields a Gemcitabine concentration of 38 mg/mL, which includes accounting for the displacement volume of the lyophilised powder. Shake to dissolve.
- Further dilution with sterile sodium chloride 9 mg/mL (0.9 %) solution for injection, can be done. Reconstituted solution is a clear colourless to light straw-coloured solution.
- Parenteral medicinal products should be inspected visually for particulate matter and discolouration prior to administration. If particulate matter is observed, do not administer.

Any unused product or waste material should be disposed of in accordance with local requirements.

s) Special precautions for storage

Unopened vial: Store below 30°C

After reconstitution and After Dilution: The reconstituted solution is stable for 36 hours at a controlled room temperature 30+/- 2 °C/75+/- 5%. Do not refrigerate as crystallization can occur.

From a microbiological point of view, the product should be used immediately. If not used immediately, in-use storage times and conditions prior to use are the responsibility of the user.

t) Shelf life

3 years

After reconstitution and After Dilution : The reconstituted solution is stable for 36 hours at a controlled room temperature 30+/- 2 °C/75+/- 5%. Do not refrigerate as crystallization can occur.

From a microbiological point of view, the product should be used immediately. If not used immediately, in-use storage times and conditions prior to use are the responsibility of the user.

u) Dosage forms and packaging available

The Gemcitabine Powder for solution for Infusion is primarily packed in 10mL tubular clear (Type –I glass) with 20mm collar flat bottom vial with 20mm Grey bromobutyl (single slotted)lyo rubber stopper and sealed with 20mm easy to open C/L Alu. Seal with matt finish plastic german blue color button.

The Gemcitabine Powder for solution for Infusion is primarily packed in 50mL tubular clear (Type –I glass) with 20mm collar flat bottom vial with 20mm Grey bromobutyl (single slotted)lyo rubber stopper and sealed with 20mm easy to open C/L Alu. Seal with matt finish plastic german blue color button.

v) Therapeutic code

L01BC05

w) Name and address of manufacturer/ product registration holder

Manufactured By:

 **SP Accure Labs Pvt. Ltd.**

Plot No.12, Biotech Park, Phase 2,
Lalgadi Malakpet, Shameerpet (M),
Medchal Dist, Telangana-500 101,
INDIA.

x) Product registration holder

Healol Pharmaceuticals Sdn.Bhd.
74-3, Jalan Wangsa Delima 6,
KLSC Wangsa Maju,
53300 Kuala Lumpur, Malaysia

y) Date of revision of PI

27, November, 2025